BASELINE → TREATMENT → FOLLOW-UP

Required safety checks and your individual disease-assessment plan

The team supplies actual dates and resolves any source ambiguity. A reference period does not replace a prescription, a local monitoring plan or an individual treatment decision.

Check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Bloods and organ safety	Confirm the BRAF V600 result, complete resection of stage III melanoma and the postoperative treatment goal. Full blood count, kidney/liver tests, ECG, echocardiogram, skin examination and eye assessment when indicated.	Blood count, kidney/liver tests monthly. ECG and echocardiogram at four weeks, then every 12 weeks and after dose modification. Regular skin reviews are required; your oncology team will confirm and record the interval. Head/neck examination every three months and chest CT every six months are specified surveillance checks; disease-response imaging follows the individual plan.	Contact the team for new symptoms; extra tests may be needed between scheduled checks.	Safety bloods do not by themselves show whether cancer is responding.	Continue skin surveillance for six months after stopping as specified by the regimen, and arrange the ongoing melanoma follow-up plan. Review symptoms, heart/eye concerns and contraception instructions after treatment.
Cancer, skin and daily function	Record diagnosis, stage, current disease sites and the baseline assessment.	Keep the source-specific skin and clinical reviews and the individually agreed response assessments.	Record new lesions, lumps, wound changes, pain and effects on usual activities.	Write the next examination or scan date and the responsible team; no universal scan or tumour-marker interval is assumed.	Follow-up depends on stage, treatment and symptoms. Your team records the skin/lymph-node examination plan, whether body or brain imaging is needed, and which service owns each appointment. A general patient booklet does not set a universal scan interval for every patient.

Check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Medicines and continuing precautions	Review all medicines and the pregnancy/fertility plan.	Review treatment taken or received, holds, adverse effects and supportive medicines.	Keep the actual urgent contacts and do not restart a held medicine yourself.	Bring the medicine list and treatment summary to other services.	Discuss fertility and pregnancy precautions before starting. Use an effective alternative to hormonal contraception during treatment and for two weeks after dabrafenib and 16 weeks after trametinib; dabrafenib can make hormonal contraception less effective. The team must confirm the method and the later applicable end date for the combination. Do not breastfeed during treatment; ask when it is safe to resume. Report a possible pregnancy promptly.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00415 v5: current regimen source · NCCN Guidelines for Patients: Melanoma, 2026 - supplied education reference

THE WHOLE TREATMENT JOURNEY

Adjuvant dabrafenib and trametinib after removal of BRAF-mutated stage III melanoma: treatment and checks

NCCP 00415b: adjuvant treatment after complete resection of stage III melanoma with a BRAF V600 mutation. This route is distinct from treatment of unresectable or metastatic melanoma.

Take dabrafenib twice daily, around 12 hours apart, and trametinib once daily, every day unless the team instructs a hold. The adjuvant course is planned for 12 months, unless stopped earlier for recurrence, side effects or another clinical reason. A four-week review is not a drug holiday or one of 12 chemotherapy cycles.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Bloods and organ safety NCCP 00415	Confirm the BRAF V600 result, complete resection of stage III melanoma and the postoperative treatment goal. Full blood count, kidney/liver tests, ECG, echocardiogram, skin examination and eye assessment when indicated.	Blood count, kidney/liver tests monthly. ECG and echocardiogram at four weeks, then every 12 weeks and after dose modification. Regular skin reviews are required; your oncology team will confirm and record the interval. Head/neck examination every three months and chest CT every six months are specified surveillance checks; disease-response imaging follows the individual plan.	Contact the team for new symptoms; extra tests may be needed between scheduled checks.	Safety bloods do not by themselves show whether cancer is responding.	Continue skin surveillance for six months after stopping as specified by the regimen, and arrange the ongoing melanoma follow-up plan. Review symptoms, heart/eye concerns and contraception instructions after treatment.
Cancer, skin and daily function NCCP 00415	Record diagnosis, stage, current disease sites and the baseline assessment.	Keep the source-specific skin and clinical reviews and the individually agreed response assessments.	Record new lesions, lumps, wound changes, pain and effects on usual activities.	Write the next examination or scan date and the responsible team; no universal scan or tumour-marker interval is assumed.	Follow-up depends on stage, treatment and symptoms. Your team records the skin/lymph-node examination plan, whether body or brain imaging is needed, and which service owns each appointment. A general patient booklet does not set a universal scan interval for every patient.

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Medicines and continuing precautions NCCP 00415	Review all medicines and the pregnancy/fertility plan.	Review treatment taken or received, holds, adverse effects and supportive medicines.	Keep the actual urgent contacts and do not restart a held medicine yourself.	Bring the medicine list and treatment summary to other services.	Discuss fertility and pregnancy precautions before starting. Use an effective alternative to hormonal contraception during treatment and for two weeks after dabrafenib and 16 weeks after trametinib; dabrafenib can make hormonal contraception less effective. The team must confirm the method and the later applicable end date for the combination. Do not breastfeed during treatment; ask when it is safe to resume. Report a possible pregnancy promptly.

What each check is for

1 Adjuvant oral treatment

12-month planned course

After eligible stage III melanoma has been completely removed, dabrafenib and trametinib are offered to reduce the chance of recurrence. The expected benefit, alternatives and treatment burden need an individual discussion; treatment cannot guarantee that melanoma will not return.

Ask / act: Confirm the current treatment phase, prescription, tests and support medicines.

Sources: NCCP 00415

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-06 · Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.40